



ಪೂಜಾ ಡಯಗ್ನೋಸ್ಟಿಕ್ಸ್ ಹೆಲ್ತ್ ಕೇರ್ ಲ್ಯಾಬ್ ಸೆಂಟರ್

POOJA DIAGNOSTICS & HEALTH CARE
RESEARCH CENTRE

Name : Indira P.S.
AGE/SEX : 46 Yrs / Female
REF BY : Self
CONTACT NO. : 9844402298

REG/LAB NO. : 25070140 / 14607
DATE OF COLLECTION : 05-07-2025 at 12:45 PM
DATE OF REPORT : 06-07-2025 at 01:02 PM



TEST PARAMETER	RESULT	REFERENCE RANGE
RHEUMATOID FACTOR	107.0 IU/mL	0 - 15 IU/mL

Interpretation:

It is more frequently present in chronic active phases of the disease than in the early or in the sub-clinical chronic phases of Rheumatoid Arthritis. RF is also found in many cases of other autoimmune diseases like systemic lupus erythematosus (SLE), scleroderma and Sjogren's syndrome. Weakly positive latex agglutination test for Rheumatoid Factor has occasionally been observed in some sera of patients with hepatitis, sarcoidosis, cirrhosis of liver, syphilis, hyper gamma globulinemia, acute bacterial and viral infection. It is almost always absent in cases of Rheumatoid fever. The Latex agglutination test does not provide final diagnosis of Rheumatoid Arthritis and therefore, it should be used only in conjunction with proper clinical evaluation.

Remarks: Note : Needs clinical correlation

C.R.P
Nephelometry

16.5 mg/L

0.2 - 6 mg/L

Remarks: Note : Needs clinical correlation

ESR

52 mm/1st hr

0 - 20 mm/1st hr

Interpretation:

ESR is non-specific marker of inflammatory process. Its main clinical utility is in monitoring the course or response to treatment of various acute and chronic disorders like hematologic diseases, malignancy, collagen vascular disorders and renal diseases.

Remarks: Note : Needs clinical correlation

Dispatched by: Raghunath s k

**** End of Report ****

Printed by: Raghunath s k on 07-07-2025 at 03:21 PM

Lab Technician


Dr. M.M. Doddikoppad M.B.B.S., MD., Dcp
Consultant Pathologist KMC Reg. No.: 14605

Speciality Hi-Tech Diagnostic Centre...

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NAME : INDIRA P. (46Y/F)
REF. BY : DR M M DODDIKOPPAD, MBBS MD
TEST ASKED : ACCP

SAMPLE COLLECTED AT :
(5770027874), HEALTHCARE CENTRE, DOOR NO
994, 4TH MAIN, P J
EXTENSION, DAVANAGERE, 577002

TEST NAME	TECHNOLOGY	VALUE	UNITS
ANTI CCP (ACCP)	C.M.I.A	298.6	U/mL

Reference Range :-

BRI	Interpretation
Negative < 5	Absence of IgG autoantibodies to cyclic citrullinated peptides (CCP)
Positive ≥ 5	Presence of IgG autoantibodies to cyclic citrullinated peptides (CCP)

Clinical Significance :

1. Anti-Cyclic-Citrullinated-Peptide (Anti-CCP) titre is used for diagnosis and monitoring of Rheumatoid Arthritis (RA).
2. RA is one of the most common systemic autoimmune diseases characterised by chronic inflammation of the synovial joints and progressive joint degeneration eventually leading to disability of affected individuals.
3. The diagnosis of RA often relies on clinical manifestations and certain non-specific laboratory tests such as rheumatoid factor (RF) and C-reactive protein (CRP), which may be present in healthy elderly persons or in patients with other autoimmune and infectious diseases.
4. Whereas, Anti-Cyclic-Citrullinated-Peptide (Anti-CCP) Antibodies hold promise for early and more accurate detection of Rheumatoid Arthritis before the disease proceeds into irreversible damage.
5. Interference with pathologic levels of nonspecific IgG can not be excluded.
6. The anti-CCP test results can be false negative in patients with hypergammaglobulinemia.
7. Results from patients suffering from this disorder should not be used for diagnostic purposes.
8. Heterophile antibodies may interfere with the test results.
9. If results are inconsistent with clinical history additional testing is suggested to confirm the results.
10. Some specimens may not dilute linearly because of heterogeneity of autoantibodies with respect to physicochemical properties.
11. HAMA (Human Anti mouse antibodies) may also interfere with the results.
12. For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

References:

- Anti-CCP Reagent Kit Insert
- Feldmann M, Brennan FM, Maini RN. Rheumatoid arthritis. Cell 1996;85:307-3102.
- Landewé RB. The benefits of early treatment in rheumatoid arthritis: confounding by indication, and the issue of timing. Arthritis Rheum 2003;48(1):1-5.

Please correlate with clinical conditions.

Method:- Fully Automated ChemiLuminescent Microparticle Immunoassay (C.M.I.A)

--- End of report ---

Sample Collected on (SCT) : 05 Jul 2025 13:40
Sample Received on (SRT) : 06 Jul 2025 00:35
Report Released on (RRT) : 06 Jul 2025 04:57
Sample Type : SERUM
Labcode : 0507044095/KAR27
Barcode : DU039471



Dr Syeda Sumalya MD(Path)

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Scan QR code to verify authenticity of reported results; active for 30 days from release time.